

SKIN & LASER

IPL & Photofacial Protocol

Filter selection, skin type exclusions, endpoints, and the melasma caution

DOCUMENT SKN-002	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO All IPL operators	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Intense pulsed light delivers broadband, non-coherent light filtered to a target range. That breadth makes it versatile and makes it the device most likely to cause pigmentary complications in the wrong patient. This protocol covers indication and filter selection, the exclusion criteria that are firm, endpoints, and series planning.

SKIN TYPE EXCLUSION

IPL is used in this practice for Fitzpatrick I–III only. Broadband output is absorbed strongly by epidermal melanin, and burns, hypopigmentation and post-inflammatory hyperpigmentation in Fitzpatrick IV–VI are well documented. Patients above the threshold are offered an appropriate laser alternative or referred — they are not treated on lower settings.

2 Indication & Filter Selection

FRAMEWORK — CONFIRM CUTOFFS AGAINST YOUR DEVICE IFU

Indication	Typical cutoff range	Target
Superficial pigmentation, lentigines, sun damage	Shorter cutoff	Melanin. Endpoint is darkening of the lesion.
Vascular — telangiectasia, diffuse erythema, rosacea	Mid-range cutoff	Oxyhemoglobin. Endpoint is vessel darkening or transient disappearance.
Mixed photodamage	Sequential or combined passes per IFU	Both chromophores. Requires more conservative parameters.
Hair reduction	Longer cutoff	Follicular melanin. See SKN-001; laser is generally preferred.
Acne — inflammatory	Per device indication	Porphyrins and sebaceous activity. Confirm the device is indicated for this use.

FILTERS ARE NOT INTERCHANGEABLE

Selecting a filter by habit rather than by indication and skin type is a common error. Confirm the cutoff, fluence and pulse structure against the IFU for every treatment, and record which filter was used.

3 Screening & Exclusions

- ☐ Fitzpatrick assessment per SKN-007. Fitzpatrick IV and above — do not treat with IPL.
- ☐ Recent sun exposure, tanning or self-tanner within the exclusion window. A tan is a contraindication regardless of baseline skin type.
- ☐ Photosensitizing medications and supplements screened.
- ☐ Melasma history — see the caution below.
- ☐ Active infection, open lesion or inflammatory dermatosis in the field.
- ☐ Recent peel, resurfacing, microneedling or injectable in the same area.
- ☐ History of keloid or hypertrophic scarring.
- ☐ Pregnancy — practice policy set by the medical director.
- ☐ Tattoos and permanent makeup in the field — avoided entirely.
- ☐ Undiagnosed or changing pigmented lesions — do not treat; refer per SKN-006.
- ☐ Test patch completed and reviewed per SKN-007.

MELASMA

Melasma is heat-sensitive and frequently worsens or rebounds after IPL, sometimes dramatically and sometimes after an initially good result. Treating melasma with IPL requires explicit medical director authorization, a documented discussion of rebound risk, and conservative parameters. Many practices exclude it entirely, which is a defensible position. Never treat melasma as though it were simple sun damage.

4 Treatment Procedure & Endpoints

- 1 **Confirm consent, screening, skin type and test patch outcome.**
- 2 **Eyewear on everyone**, appropriate to the emission range. Patient eye shields for facial treatment. See SKN-008.
- 3 **Cleanse thoroughly** — all makeup, sunscreen and residue removed. Skin dry.
- 4 **Apply coupling gel** at the thickness specified by the IFU. Insufficient gel is a direct cause of burns.
- 5 **Confirm filter, fluence, pulse duration and delay** against the chart and IFU.
- 6 **Maintain full crystal contact** with the skin. Partial contact concentrates energy at the edge and burns.
- 7 **Treat in a planned pattern** with the overlap specified by the IFU. Avoid stacking pulses on one site unless the IFU directs it.
- 8 **Watch the endpoint continuously** and stop on any warning sign.
- 9 **Cool the area** after treatment and apply a bland emollient.
- 10 **Written aftercare** and warning signs supplied with contact instructions.

- 11** **Record** filter, fluence, pulse settings, passes, areas, endpoint and response.

Observation	Interpretation
Mild erythema; pigmented lesions darken	Desired response. Continue.
Vessels darken or briefly disappear	Desired vascular endpoint.
Immediate greying or whitening	Epidermal injury. Stop immediately.
Blistering during or immediately after	Burn. Stop, cool, follow SKN-009.
Severe pain disproportionate to the setting	Excessive energy or poor coupling. Stop.
Rectangular imprints matching the crystal footprint	Overtreatment or poor overlap. Stop and reassess technique.

5 Aftercare & Series Planning

- Darkened pigment typically develops a coffee-ground appearance and sheds over days — warn the patient in advance or they will believe something went wrong.
- No picking, scrubbing or exfoliating the treated area until shedding is complete.
- Broad-spectrum sun protection daily, and strict sun avoidance between sessions. A patient who tans between treatments cannot be treated on schedule.
- No heat exposure — sauna, hot yoga, steam — for the period specified in aftercare.
- No active skincare such as retinoids or acids until the skin has settled.
- Series length and interval set with the medical director by indication.
- Reassess skin type, tanning status and medications before every session in a series, not only at the first.
- Maintenance planning discussed at the outset, since photodamage recurs with exposure.

6 Quick Reference

QUICK REFERENCE CARD

Before every IPL pulse

1. **Fitzpatrick I–III only.** IV and above → alternative or referral.

2. **Any tan is a contraindication.** Re-screen at every session.

3. **Melasma?** Medical director authorization and rebound discussion, or decline.

4. **Filter chosen by indication,** confirmed against the IFU.

5. **Generous coupling gel. Full crystal contact.** Both cause burns when neglected.

6. **Eyewear and eye shields** on everyone.

7. **Greying, whitening or blistering → stop.**

8. **Warn about the coffee-ground shedding** before they see it.

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME

LOCATION / SITE

MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS

LICENSE NUMBER & STATE

SIGNATURE

DATE ADOPTED / NEXT REVIEW DATE

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